

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

January 8, 2024

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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Deputy Assistant Director of Clinical Services/Medical Director
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FROM:

CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Chief, Investigations Unit
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LCDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator, Investigations Unit
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SUBJECT:

Mortality Review – Report of Findings
Subash SHRESTHA, (b)(6),(b)(7)(C)

Executive Summary:

On November 13, 2023, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detained noncitizen Subash SHRESTHA. Mr. SHRESTHA, a 34-year-old Nepali male, was in ICE custody from September 9 to November 13, 2023, and assigned to Karnes County Immigration Processing Center (KCIPC), in Karnes City, Texas, on the date of his death. Mr. SHRESTHA's initial cause of death report ruled his cause of death asphyxia by hanging; his final autopsy report is pending.

Prior to ICE custody, on September 2, 2023, Mr. SHRESTHA presented to the United States Border Patrol (USBP) near Lukeville, Arizona, and was issued a Notice and Order of Expedited

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Removal, Form I-860, charging inadmissibility pursuant to section 212(a)(7)(A)(i)(I) of the Immigration and Nationality Act (INA), as a noncitizen in the United States without an immigrant visa. On September 9, 2023, USBP transferred Mr. SHRESTHA to ICE Enforcement and Removal Operations (ERO) San Antonio, and ICE housed him at KCIPC.

On September 9, 2023, during his intake screening, Mr. SHRESTHA denied a history of any medical or mental health conditions, illicit drug use, or hospitalizations, and medical staff cleared him for general population.

Between September 28 and November 10, 2023, the KCIPC medical and mental health staff evaluated Mr. SHRESTHA intermittently for mental health concerns related to his immigration case. During this time, ICE deportation officers (DOs) also educated Mr. SHRESTHA about the status of his immigration case and the immigration process.

On November 13, 2023, at approximately 11:56 a.m., correctional staff responded to an unresponsive Mr. SHRESTHA in his room, called a medical emergency via the facility radio, evacuated Mr. SHRESTHA's roommates from his room, and initiated cardiopulmonary resuscitation (CPR). At approximately 11:58 a.m., medical staff arrived to Mr. SHRESTHA's room and continued life-saving efforts until emergency medical services (EMS) personnel arrived at 12:07 p.m. At 12:48 p.m., EMS personnel pronounced Mr. SHRESTHA deceased.

Mortality Finding:

As a result of Mr. SHRESTHA's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined that Mr. SHRESTHA's medical care at KCIPC was not provided within the safe limits of practice. Specifically, the MRC determined: 1) A behavioral health provider (BHP) did not consistently evaluate Mr. SHRESTHA within 72 hours of his referral to mental health; 2) KCIPC's medical staff did not respond to Mr. SHRESTHA's emergency with all necessary equipment; and 3) licensed practical nurses (LPN) acted outside their scope of practice by performing assessments independently and without evidence of oversight from a higher-level provider.

The MRC convened on December 12, 2023, and consisted of the following IHSC members: Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F), medical doctor (MD), South Texas ICE Processing Center Clinical Director, physician reviewer; Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F), physician assistant, chief advanced practice provider (APP), APP reviewer; CAPT (b)(6),(b)(7)(C),(b)(7)(F)

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eastern regional nurse manager, nurse reviewer; and LCDR (b)(6),(b)(7)(C),(b)(7)(F) psychologist, behavioral health clinical consultant, BHP reviewer.

The following summary of events was based on: 1) Mr. SHRESTHA's KCIPC medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. SHRESTHA's KCIPC detention file; 5) Mr. SHRESTHA's initial cause of death report; 6) video footage from November 13, 2023; 7) an on-site review and staff interviews conducted by Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F), LCDR (b)(6),(b)(7)(C),(b)(7)(F) and LCDR (b)(6),(b)(7)(C),(b)(7)(F) at KCIPC from December 4 to 6, 2023; and 8) applicable KCIPC policies and procedures, and ICE Detention Standards.

Summary of Events

On September 9, 2023, a registered nurse (RN) completed Mr. SHRESTHA's intake screening. Mr. SHRESTHA denied a history of any medical or mental health conditions, illicit drug use, or hospitalizations. The RN documented a normal exam, submitted a routine physician and dental referral, and cleared him for general population.

On September 17, 2023, an RN completed Mr. SHRESTHA's initial physical examination, documented his denial of any medical or mental health conditions and a normal exam, and returned him to general population.

On September 28, 2023, an LPN evaluated Mr. SHRESTHA for abdominal and epigastric pain after eating, documented an exam within normal limits, issued an antacid and omeprazole (reduces gastric acid) 20 milligrams, one tablet, by mouth, twice daily per a provider's verbal order, and educated him on diet restrictions and relief measures.

On September 30, 2023, an LPN evaluated Mr. SHRESTHA for complaints of upper abdominal and chest pain, completed an electrocardiogram [(ECG); measures electrical activity of the heart], which showed normal results, documented positive abdominal guarding, skin diaphoretic, normal vital signs, except an elevated blood pressure of 141/88 millimeters of mercury [(mmHg); normal range: 90/60 – 120/80mmHg], and provided one dose of an antacid. After taking the medication, Mr. SHRESTHA reported feeling better and stated he was "feeling stressed" due to being at the facility and requested to see mental health. The LPN submitted a routine mental health referral and instructed Mr. SHRESTHA to return to the clinic as needed.

On October 3, 2023, a mental health case worker (MHCW) completed Mr. SHRESTHA's mental health encounter. Mr. SHRESTHA reported missing his family and worrying about his immigration case, denied thoughts of self-harm and suicidal ideations, and declined to speak with a BHP. The MHCW documented a normal exam and completed a refusal form.

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On October 6, 2023, an LPN evaluated Mr. SHRESTHA for trouble sleeping and feeling anxious. The LPN documented a normal exam and submitted a routine mental health referral.

- On this same date, an MHCW completed Mr. SHRESTHA's mental health encounter. Mr. SHRESTHA reported his inability to sleep due to worrying about his immigration case and denied suicidal ideations. The MHCW documented Mr. SHRESTHA's appearance as appropriate and mood euthymic, and informed him of an upcoming BHP appointment on October 9, 2023.

On October 9, 2023, a BHP attempted to complete Mr. SHRESTHA's mental health evaluation. Mr. SHRESTHA reported exercising, which helped improve his sleep, denied suicidal or homicidal ideations, and declined further mental health evaluation/services. The BHP completed a refusal form and returned Mr. SHRESTHA to general population.

On October 13, 2023, an LPN evaluated Mr. SHRESTHA after being escorted to medical for complaints of abdominal and chest pain. Mr. SHRESTHA admitted to not taking his omeprazole as prescribed and reported feeling anxious and stressed. The LPN completed an ECG, which showed sinus bradycardia (heart rate <60 beats per minute), otherwise normal results, documented an exam within normal limits, placed Mr. SHRESTHA's medication on pill line, and referred him to physician sick call and a BHP.

On October 14, 2023, a physician completed Mr. SHRESTHA's follow-up evaluation, documented "physical exam unremarkable," normal vital signs, except a slightly elevated blood pressure of 135/89 mmHg, and instructed Mr. SHRESTHA to follow up as needed.

On October 19, 2023, an RN evaluated Mr. SHRESTHA for a headache, documented an exam within normal limits, and provided him pain medication per a provider's order.

On October 21, 2023, Mr. SHRESTHA reported to sick call requesting help with his immigration case, reported feeling mild stress, eating well, and exercising to help decrease his stress, and denied suicidal ideations. An RN submitted a next-day mental health referral and alerted an ICE officer, who spoke with Mr. SHRESTHA about his immigration case.

On October 22, 2023, an LPN completed Mr. SHRESTHA's sick-call evaluation for his complaints of stress and headaches, informed him of his upcoming BHP appointment, administered a dose of his ordered pain medication, and returned him to general population.

On October 24, 2023, a BHP completed Mr. SHRESTHA's initial mental health evaluation. Mr. SHRESTHA reported sleeping, exercising, and eating well, and denied a history of mental health conditions, suicide attempts, or current suicidal ideations. The BHP documented normal exam findings, psychiatry referral not necessary, and no increased suicide risk, scheduled a follow-up

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appointment in two weeks, and recommended he attend stress management group.

October 27, 2023, an LPN evaluated Mr. SHRESTHA for complaints of chest pain, a headache for three days, and his request to see a mental health provider. The LPN documented normal vital signs and exam findings and submitted a mental health referral.

- On this same date, an MHCW evaluated Mr. SHRESTHA due to the LPN's request and offered him an appointment with a BHP. Mr. SHRESTHA reported experiencing stress related to his immigration case, exercising, and practicing breathing exercises, denied suicidal ideations, and agreed to wait for his scheduled BHP appointment on November 1, 2023. The MHCW completed a refusal form.

On October 30, 2023, a correctional officer escorted Mr. SHRESTHA to the mental health clinic. An MHCW evaluated Mr. SHRESTHA, documented his tearful, anxious appearance and denial of suicidal ideations, and reminded him of his upcoming BHP appointment.

On October 31, 2023, a case manager escorted Mr. SHRESTHA to the mental health clinic. An MHCW evaluated Mr. SHRESTHA, who appeared distressed and poorly groomed, reported stress related to the timeline of his immigration case, and denied suicidal ideations. The MHCW again reminded him of his upcoming BHP appointment.

On November 1, 2023, a BHP completed Mr. SHRESTHA's mental health evaluation, and documented Mr. SHRESTHA's normal appearance, a normal exam, and his report of reading, exercising, and practicing his faith to cope. The BHP documented an assessment of psychosocial stress, recommended Mr. SHRESTHA attend stress management group, taught him a breathing technique, and scheduled a 14-day follow-up appointment.

On November 2, 2023, correctional staff escorted Mr. SHRESTHA to the mental health clinic and informed an MHCW of his frequent complaints of anxiety, chest pains, and panic attacks. The MHCW completed Mr. SHRESTHA's evaluation, documented his tearful but calm appearance and denial of suicidal ideations, encouraged him to continue with his coping skills, and instructed him to contact ICE regarding his immigration case.

On November 3, 2023, an LPN evaluated Mr. SHRESTHA for his complaint of chest pain. Mr. SHRESTHA denied suicidal ideations or a history of taking anxiety medications but requested to have something prescribed. The LPN documented a normal examination, except an anxious mood and tearful affect, submitted an urgent mental health referral, and administered his prescribed pain medication for his headache.

On November 6, 2023, an advanced practice provider (APP) evaluated Mr. SHRESTHA due to a call for medical assistance related to Mr. SHRESTHA experiencing dizziness. Mr. SHRESTHA reported feeling tired, stumbling, and falling in his room, denied passing out, and requested to

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return to his room to sleep. The APP documented a normal blood glucose and vital signs, and instructed Mr. SHRESTHA to return to the clinic as needed.

On November 7, 2023, ICE DOs spoke to Mr. SHRESTHA about his immigration case after receiving a staff detainee communication asking for help with his case and his “mind problem.” The ICE DOs escorted Mr. SHRESTHA to the mental health clinic; however, there is no documentary evidence a mental health provider evaluated Mr. SHRESTHA that day.

On November 9, 2023, an RN completed Mr. SHRESTHA’s sick-call evaluation for mild left knee pain, documented a normal exam and vital signs, and provided him muscle rub cream.

On November 10, 2023, a psychiatrist completed Mr. SHRESTHA’s initial psychiatric evaluation via telemedicine. Mr. SHRESTHA reported “doing alright overall,” but voiced concerns about his immigration case. Mr. SHRESTHA denied past suicide attempts, past self-injurious behavior, and current or past suicidal ideations. The psychiatrist diagnosed Mr. SHRESTHA with adjustment disorder with anxiety, documented appropriately groomed and dressed, anxious mood, appropriate affect, good insight and judgment, and no psychiatric follow-up needed, and recommended mental health follow-up and supportive therapy as needed.

On November 13, 2023, at approximately 11:56 a.m., a noncitizen housed in Mr. SHRESTHA’s room found him hanging from a sheet in the room’s bathroom. The noncitizen verbally notified the other noncitizens housed in the same room, who assisted the noncitizen to remove the sheet, move Mr. SHRESTHA out of the bathroom and into the room, and notify the correctional staff. The correctional staff responded to Mr. SHRESTHA’s room, observed a noncitizen extend his arm out of the bathroom and toss a sheet under the sink, called a medical emergency via the facility radio, evacuated the noncitizens from the room, and initiated CPR.

- At approximately 11:57 a.m., a correctional supervisor directed control to call 911.
- At approximately 11:58 a.m., a responding RN arrived at Mr. SHRESTHA’s room and observed correctional officers performing CPR on Mr. SHRESTHA, who was unresponsive on the floor. Additional medical staff arrived with the emergency equipment, including the automated external defibrillator (AED). An LPN applied the AED, the AED analyzed and indicated no shock advised, and medical staff continued CPR. Throughout the duration of the response, the AED did not advise or deliver a shock.
- At approximately 12:07 p.m., EMS personnel arrived at Mr. SHRESTHA’s room, assumed care of Mr. SHRESTHA, and continued CPR and life-saving measures until the EMS medical director instructed them to cease all life-saving efforts.
- At 12:48 p.m., EMS personnel pronounced Mr. SHRESTHA deceased.

The initial cause of death report revealed Mr. SHRESTHA’s cause of death was asphyxia by hanging. The coroner determined the manner of death as suicide. Mr. SHRESTHA’s final autopsy report and the results of his toxicology studies are pending at the time of this report.

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Psychologist and Behavioral Health Clinical Consultant LCDR (b)(6),(b)(7)(C) performed Mr. SHRESTHA's psychological autopsy (procedure for analyzing the cause of death by reconstructing what the person thought, felt, and did before death, based on gathered information). The final psychological autopsy report is pending.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, and the National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) Mental health staff frequently evaluated Mr. SHRESTHA; 2) BHPs review all intake screenings to ensure all noncitizens requiring an initial mental health assessment receive an evaluation; 3) The responding correctional officers immediately administered CPR, called for an AED, and called for EMS before medical staff arrived on scene; 4) Correctional, ICE, and medical staff (non-mental health) regularly referred Mr. SHRESTHA to mental health for concerning behavior; 5) The staff who participate in quarterly man-down drills provide feedback on the drill and response through written surveys.

Weaknesses

1. Inappropriate mental health care.

- On November 3, 2023, an LPN evaluated Mr. SHRESTHA and submitted an urgent mental health referral; however, a behavioral health provider did not evaluate Mr. SHRESTHA within 72 hours. A behavioral health provider did not evaluate Mr. SHRESTHA until the psychiatrist evaluated him on November 10, 2023.
- On November 7, 2023, ICE DOs spoke to Mr. SHRESTHA after receiving a staff detainee communication in which he expressed mental distress. After speaking with Mr. SHRESTHA, the ICE DOs escorted Mr. SHRESTHA to the mental health clinic, but there is no documentary evidence to confirm a BHP evaluated him that day.
- (b)(7)(E)

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section O. *Mental Health Program*.
- NCCHC 2018 standards J-A-01, *Access to Care*.
- NCCHC 2018 standards J-F-03, *Mental Health Services*.
- KCIPC Policy: 4.1.3, *Detainee Medical Services*, 2023.

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2. Inappropriate emergency care.

- On November 13, 2023, medical staff were notified correctional staff initiated CPR, but they responded to Mr. SHRESTHA's medical emergency without necessary medical equipment, specifically without a bag valve mask.
- While KCIPC conducts man-down drills quarterly, there is no documentary evidence medical staff who were not present for the drills review the critiques.

Applicable standards of care for this finding:

- NCCHC 2018 standards: J-D-07, *Emergency Services and Response Plan*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- KCIPC Policy: 608-A, *Emergency Services*, 2023.

3. Inappropriate medical care.

- On October 6, 13, 22, 27, and 28, and November 3, 2023, LPNs completed Mr. SHRESTHA's sick-call evaluations and developed plans of care; however, there is no evidence the LPNs reported their assessment findings to a higher-level provider.

Applicable standards of care for this finding:

- NCCHC 2018 standards: J-E-08, *Nursing Assessment Protocols and Procedures*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section B. *Designation of Authority*.
- Texas Board of Nursing 2022: Position Statement 15.27, *The Licensed Vocational Nurse Scope of Practice*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, KCIPC's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.